

85%

of C-sections are performed for **one of four reasons:** previous C-sections, lack of progress in labor, fetal distress, or breech birth.

Q What happens before surgery? Do I need to prepare myself?

For both planned and emergency C-sections, there is a set of procedures to follow.

» **Written consent:** you will need to sign a consent form to say that you agree to the surgery, understand the risks, and allow the doctors to act in your best medical interests.

» **Meeting the anesthesiologist:** If there's time, the consultant who will administer the drugs will explain what form of anesthetic you will have and what its effects will be, both during and after the surgery.

» **Preparing yourself:** you'll need to remove any jewelry, makeup, nail polish, and fake nails (so that the anesthesiologist can monitor your skin and nail color to check oxygen levels).

» **Being prepared by the medical team:** a nurse will shave your pubic hair around the intended incision site. For a planned C-section, you will be given a blood test for anemia (which increases risk of blood loss) and blood type, in case a transfusion is necessary during surgery. You'll be attached to an arm cuff and chest electrodes to measure your blood pressure and heart rate throughout. You'll be given antibiotics to reduce the risk of postoperative infection, and low blood pressure medication if you need it. For general anesthesia you will be given antacids and medication to prevent nausea and vomiting.

» **Immediately before surgery:** finally, you will be given your anesthesia, and a catheter will be inserted into your ureter, to provide bladder control during surgery.

Q Why might I need a cesarean section?

There are many reasons for a cesarean section, but usually they are used to prevent or solve health risks to you or your baby. Nearly half are elective, planned in advance for known medical problems or by request. The remainder are advised when issues arise during labor and a quick delivery is needed.

REASONS FOR PLANNED CESAREAN SECTION

» **Repeat C-section** could be proposed by your obstetrician if your medical history presents risk factors, such as complications with the previous procedure or a high chance of old scars tearing in a vaginal birth.

» **Breech presentation or transverse lie** (when your baby is in a bottom down or sideways position in the uterus) might need a C-section if the baby can't be turned manually using external cephalic version.

» **Preeclampsia**, a condition that can develop during pregnancy, could make the strain of vaginal birth dangerous for you.

» **Low-lying placenta** can block the passage of your baby or make heavy bleeding likely.

» **Twins or multiples** often require a C-section, although vaginal delivery is possible in twins if the first baby is positioned head down.

» **Some heart conditions** make vaginal birth unsafe for the mother.

» **Maternal requests** for a C-section are sometimes made if a previous birth was difficult.

REASONS FOR UNPLANNED CESAREAN SECTION

» **Lack of progress in labor** can happen if your cervix hasn't opened enough after a long, slow labor, or contractions are too weak. Although not life threatening, this is the most common reason for unplanned C-sections.

» **Fetal distress** is sometimes detected during labor. Doctors monitor the baby's heart rate as standard practice for all births.

» **Unsuccessful assisted delivery** (using forceps or vacuum) can be followed by a C-section.

» **Placental abruption** is when the placenta comes away from the wall of the uterus either before or during labor. It is a rare occurrence but necessitates an emergency C-section.

» **Uterine rupture** occurs if a scar from a previous C-section tears during labor. It happens in less than 1 percent of women with prior C-sections. Your doctor will assess the risk of rupture before planning a vaginal birth.

» **Umbilical cord prolapse** is another very rare event. If the umbilical cord slips down the cervix ahead of your baby, it can get compressed and affect the baby's oxygen supply.

Q Can I stay awake during a C-section?

In most cases, you can stay awake because you will be given either spinal anesthesia or an epidural; both numb the lower body without making you unconscious. Spinal anesthesia (or spinal block) is a single injection that is used only in the event of surgery. With these forms of anesthetic, you won't feel any pain during surgery, but you might feel a bit of pushing and pulling in your abdomen. It helps to focus on

music—arrange to take some music of your choice to be played in the operating room. However, if a quick delivery is vital, your doctor might use general anesthesia, which is faster to administer, but means that you will be fully asleep. General anesthesia is only used if the benefits outweigh the risks—it can cross the placenta and make your baby drowsy, but studies show there are no long-lasting effects on the baby. The surgery usually takes 40–50 minutes; maybe longer if you've had a C-section before.